

PROVISION OF PSYCHOSOCIAL HEALTH SUPPORT

Unit of learning code HE/CU/TT/CC/07/6/A

Related Unit of Competency in Occupational Standard Provide Psychosocial Health Support Services

Introduction to the unit of learning

This Unit describes the competencies required to provide psychosocial health support. It involves identifying and assessing patients for psychosocial healthcare, caring for patients with psychosocial health needs, linking same patients to appropriate psychosocial healthcare, following-up on psychosocial healthcare support and documenting psychosocial health support services.

Summary of Learning Outcomes

- 1 Identify and assess patients for psychosocial healthcare need
- 2 Care for patients with psychosocial health needs
- 3 Follow up on psychosocial healthcare support
- 4 Document psychosocial healthcare support

Learning Outcome 1: Identify and assess patients for psychosocial healthcare need.

Introduction to the learning outcome

This learning outcome specifies the content of competencies required to identify and assess patients for psychosocial healthcare need. It includes Basic counselling skills, Identification of psychosocial needy patients, Psychosocial health needs, Components of psychosocial healthcare and Patient assessment of psychosocial needs.

Performance Standard

1.1 Patient for psychosocial healthcare support is identified as per the WHO standards and Psychosocial Healthcare Standards.

1.2 Rapport is created between the healthcare giver and the patient as per WHO and Psychosocial Healthcare Standards.

1.3 Patient is assessed for psychosocial healthcare need as per WHO standards and work place policy.

1.4 Nature and extent of psychosocial healthcare need is analysed in accordance with Psychosocial Healthcare Standards and WHO standards.

7.3 Information Sheet

Definition of terms

Human Psychology: The scientific study of behavior and mental processes.

Human development: The scientific study of the changes that occur in people as they age from conception until death.

Development: refers to systematic continuities and changes in the individual that occur between conception (when the father's sperm penetrates the mother's ovum, creating a new organism) and death.

Counselling: refers to a form of confidential helping which values and seeks to elicit each client's innate internal resources, coping abilities and strengths.

Basic Human Psychology

Human Psychology refers the scientific study of behavior and mental processes. Behavior includes all of our outward or overt actions and reactions, such as talking, facial expressions, and movement. The term mental processes refer to all the internal, covert activity of our minds, such as thinking, feeling, and remembering. Why "scientific"? To study behavior and mental processes in both animals and humans, researchers must observe them. Whenever a human being observes anyone or anything, there's always a possibility that the observer will see only what he or she expects to see. Psychologists don't want to let these possible biases cause them to make faulty observations. They want to be precise, and to measure as carefully as they can—so they use the scientific method to study psychology.

The major goals of studying human psychology include:

Description human behavior

Explanation human behavior

Prediction human behavior and

Control human behavior

Understanding behavior is dependent on the particular view point taken, in other words, how any behavior is interpreted will depend on the theory used to explain it.

For example, a behaviorist (more on this in later pages) would conclude that, for example, smoking is encouraged by the approval of significant people in one's life, and it persists because of this continuing approval (before it becomes an addiction), whereas a Freudian might conclude that smoking is a behavior which points to the person having not coped adequately with a particular conflict early in life. The former concludes that behavior will be

repeated when it is approved, whereas the latter places weight on experiences in early life as having influences on how the person behaves in later life.

The five major perspectives are:

Biological Perspective: From this perspective psychologists primarily study the brain and nervous system with a view to understanding how these bodily structures influence behavior. We act in the world using our bodies. By that I mean we move, walk, see, taste, hear, touch and smell with our sensory systems. We think and reason about the world using our senses, which are mediated through the body, through the nervous system and neuromuscular equipment. This means that psychology cannot be disentangled from biology. The study of the brain, nervous system and hormones helps with an understanding of how we learn about the world, what we want in and from the world, and why we act in the ways that we do. For example, we know that the sensory system (sight, hearing, touching, tasting, smelling) is the primary means by which we receive information about the world, from birth.

Behavioral perspective: This is the study of learning by observing the direct effects of external environmental stimuli on behavior and behavior change. It refers to the study of behavior change. It is based on the assumption that behavior change signifies that learning has taken place. Behaviorism's greatest impact has been the development of therapies for fears and phobias, anxiety disorders, the management of unwanted or challenging behaviors and its contribution to Cognitive Behavioral Therapy.

Psychoanalytic perspective: The study of the influence of childhood experiences on current psychological or emotional states. Psychoanalysis was founded by Sigmund Freud (a medical doctor) as a method of inquiry, a theory of mind and a mode of treatment for complex psychological problems.

Cognitive perspective: This is the study of cognition mental processes including memory, perception, information processing, psychophysiology, psychoneuroimmunology and social cognition. Cognitive psychology is concerned with thought processes. It is originally based on experimental studies of memory, perception and information processing.

Humanistic perspective: This is the subjective study of human experience. Humanistic psychologists are concerned with individual perceptions and interpretations which are influenced by social and cultural meanings and past experiences. Therefore individual perceptions may change over time and vary in different social and cultural settings. The main focus is the individual's sense of self.

Professionals in the field of psychology

Psychology is a large field, and the many professionals working within it have different training, different focuses, and may have different goals from the typical psychologist.

Psychologists undergo intense academic training, learning about many different areas of psychology before choosing a specialization.

Clinical psychologists: aim to reduce psychological distress and enhance and promote psychological well being. They further deal with assessment and diagnosis of psychological disorders.

Educational psychology: Educational psychology is the study of how humans learn in educational settings, the effectiveness of educational interventions, the psychology of teaching.

Health psychologists: apply psychological research and methods to the strategic prevention and management of disease, the promotion and maintenance of health, the identification of psychological factors that contribute to physical illness, and the formulation of health policy.

Counseling psychologists: apply psychology to working collaboratively with people across a diverse range of human problems. This includes helping people manage difficult life events such as bereavement, past and present relationships and mental health problems such as depression.

Forensic psychologists: Forensic psychologists apply psychological principles to understand the behavior of judges, attorneys, courtroom juries, and others in the criminal justice system.

Personality psychologists: These psychologists study people and the differences among them. The goal is to develop theories that explain the psychological processes of individuals, and to focus on individual differences

Sports psychologists: This field studies the psychological aspects of sports behavior. The goal is to understand the psychological factors that influence performance in sports, including the role of exercise and team interactions.

Biopsychology and neuroscience: This field examines the physiological bases of behavior in animals and humans by studying the functioning of different brain areas and the effects of hormones and neurotransmitters on behavior.

Developmental psychology: Mainly focusing on the development of the human mind through the life span, developmental psychology seeks to understand how people come to perceive, understand, and act within the world and how these processes change as they age.

Social psychology: Social psychology is the study of the nature and causes of human social behavior, with an emphasis on how people think towards each other and how they relate to each other.

Basic Human development and Behavior

Beginning to understanding how we come to be the people we are is a critical step in understanding ourselves as we are today, and who we may become as we grow older. From the moment of conception, each of us is headed down a pathway of change, influenced by our biology, environment, and social interactions, to a final destination that is the same for all of us.

In development, by describing changes as “systematic,” we imply that they are orderly, patterned, and relatively enduring, so temporary mood swings and other transitory changes in our appearances, thoughts, and behaviours are therefore excluded.

We are also interested in continuities in development, or ways in which we remain the same or continue to reflect our past.

If development represents the continuities and changes an individual experiences from “womb to tomb,” *developmental sciences* refer to the study of these phenomena and is a multidisciplinary enterprise. Although developmental psychology is the largest of these disciplines, many biologists, sociologists, anthropologists, educators, physicians, and even historians share an interest in developmental continuity and change, and have contributed in important ways to our understanding of both human and animal development.

Because the science of development is multidisciplinary, we use the term *developmentalist* to refer to any scholar—regardless of discipline—who seeks to understand the developmental process.

The second critical developmental process is *learning*—the process through which our experiences produce relatively permanent changes in our feelings, thoughts, and behaviours. Let’s consider a very simple example. Although a certain degree of physical maturation is necessary before a primary school child can become reasonably proficient at dribbling a basketball, careful instruction and many, many hours of practice are essential if this child is ever to approximate the ball-handling skills of a professional basketball player. Many of our abilities and habits do not simply unfold as part of maturation; we often learn to feel, think, and behave in new ways from our observations of and interactions with parents, teachers, and other important people in our lives, as well as from events that we experience. This means that we change in response to our environments—particularly in response to the actions and reactions of the people around us.

Basic Characteristics of Development

Continual and Cumulative Process: Although no one can specify precisely what adulthood holds in store from even the most meticulous examination of a person’s childhood, developmentalists have learned that the first 12 years are extremely important for setting the stage for adolescence and adulthood. Who we are as adolescents and adults also depends on the experiences we have later in life. Obviously, you are not the same person you were at age 10 or even at age 15. You have probably grown somewhat, acquired new academic skills,

and developed very different interests and aspirations from those you had as a standard 5. And the path of such developmental change stretches ever onward, through middle age and beyond, culminating in the final change that occurs when we die. In sum, human development is best described as a continual and cumulative process.

Holistic Process: humans are physical, cognitive, and social beings, and each of these components of self depends, in part, on changes taking place in other areas of development. When we say development is holistic, we mean that we develop physically, psychologically, emotionally, socially and cognitive.

Historical/Cultural Context: Development is also influenced by societal changes: historical events such as wars, technological breakthroughs such as the development of the Internet, and social causes such as the gay and lesbian movement. Each generation develops in its own way, and each generation changes the world for succeeding generations. So we should not automatically assume that developmental patterns observed in North American or European children (the most heavily studied populations) are optimal, or even that they characterize persons developing in other eras or cultural settings.

Domains of development

There are four major domains of development. They include:

Physical domain: This refers to the development of physical characteristics such as height, weight, skull formation, motor development

Cognitive domain: This refers to the development of our mental processes such as information processing, thinking, language, reasoning, moral development

Personality Domain: This refers to the development of our enduring patterns of behavior including our temperament

Psycho-social Domain: This refers to the development of how we form relationships and our sense of being male or female.

Lifespan stages of development

Human beings go through 8 major stages of lifespan development. These stages include:

Prenatal Development: This is the stage between conception and birth. This stage consists of three stages;

Geminal stage: Lasts for 2 weeks after fertilization of the ovum by the sperm when a zygote is formed and the process of cell division begins as the zygote moves to implant itself on the uterine wall.

Embryonic stage: Lasts upto 6 weeks after conception. The process of cell division continues rapidly as the various organs of the human body begin to form.

Fetal stage: This stage lasts between 6 weeks to birth (40 weeks). During this stage, most of the body organs have been formed are continuing to form and function.

Infancy and toddlerhood: The first year and a half to two years of life are ones of dramatic growth and change. A newborn, with many involuntary reflexes and a keen sense of hearing but poor vision, is transformed into a walking, talking toddler within a relatively short period of time. Caregivers similarly transform their roles from those who manage feeding and sleep schedules to constantly moving guides and safety inspectors for mobile, energetic children. Brain development happens at a remarkable rate, as does physical growth and language development.

Early Childhood stage: Early childhood is also referred to as the preschool years, consisting of the years that follow toddlerhood and precede formal schooling, roughly from around ages 2 to 5 or 6. As a preschooler, the child is busy learning language (with amazing growth in vocabulary), is gaining a sense of self and greater independence, and is beginning to learn the workings of the physical world. This knowledge does not come quickly, however, and preschoolers may initially have interesting conceptions of size, time, space and distance, such as demonstrating how long something will take by holding out their two index fingers several inches apart.

Middle Childhood stage: The ages of 6-11 comprise middle childhood and much of what children experience at this age is connected to their involvement in the early grades of school. Now the world becomes one of learning and testing new academic skills and assessing one's abilities and accomplishments by making comparisons between self and others. Schools participate in this process by comparing students and making these comparisons public through team sports, test scores, and other forms of recognition. The brain reaches its adult size around age seven, but it continues to develop. Growth rates slow down and children are able to refine their motor skills at this point in life. Children also begin to learn about social relationships beyond the family through interaction with friends and fellow students; same-sex friendships are particularly salient during this period.

Adolescence stage: Adolescence is a period of dramatic physical change marked by an overall physical growth spurt and sexual maturation, known as puberty; timing may vary by gender, cohort, and culture. It is also a time of cognitive change as the adolescent begins to think of new possibilities and to consider abstract concepts such as love, fear, and freedom. Ironically, adolescents have a sense of invincibility that puts them at greater risk of dying from accidents or contracting sexually transmitted infections that can have lifelong consequences. Research on brain development helps us understand teen risk-taking and impulsive behavior. A major developmental task during adolescence involves establishing one's own identity. Teens typically struggle to become more independent from their parents. Peers become more important, as teens strive for a sense of belonging and acceptance;

mixed-sex peer groups become more common. New roles and responsibilities are explored, which may involve dating, driving, taking on a part-time job, and planning for future academics.

Young adulthood stage: Late teens, twenties, and thirties are often thought of as early adulthood (students who are in their mid to late 30s may love to hear that they are young adults!). It is a time when we are at our physiological peak but are most at risk for involvement in violent crimes and substance abuse. It is a time of focusing on the future and putting a lot of energy into making choices that will help one earn the status of a full adult in the eyes of others. Love and work are the primary concerns at this stage of life. In recent decades, it has been noted (in the U.S. and other developed countries) that young adults are taking longer to “grow up.” They are waiting longer to move out of their parents’ homes, finish their formal education, take on work/careers, get married, and have children. One psychologist, Jeffrey Arnett, has proposed that there is a new stage of development after adolescence and before early adulthood, called “emerging adulthood,” from 18 to 25 (or even 29) when individuals are still exploring their identities and don’t quite feel like adults yet. Cohort, culture, time in history, the economy, and socioeconomic status may be key factors in when youth take on adult roles.

Middle adulthood stage: The late thirties (or age 40) through the mid-60s are referred to as middle adulthood. This is a period in which physiological aging that began earlier becomes more noticeable and a period at which many people are at their peak of productivity in love and work. It may be a period of gaining expertise in certain fields and being able to understand problems and find solutions with greater efficiency than before. It can also be a time of becoming more realistic about possibilities in life; of recognizing the difference between what is possible and what is likely. Referred to as the sandwich generation, middle-aged adults may be in the middle of taking care of their children and also taking care of their aging parents. While caring about others and the future, middle-aged adults may also be questioning their own mortality, goals, and commitments, though not necessarily experiencing a “mid-life crisis.”

Late adulthood stage: This period of the lifespan, late adulthood, has increased in the last 100 years, particularly in industrialized countries, as average life expectancy has increased. Late adulthood covers a wide age range with a lot of variation, so it is helpful to divide it into categories such as the “young old” (65-74 years old), “old old” (75-84 years old), and “oldest old” (85+ years old). The young old are similar to middle-aged adults; possibly still working, married, relatively healthy, and active. The old old have some health problems and challenges with daily living activities; the oldest old are often frail and in need of long term care. However, many factors are involved and a better way to appreciate the diversity of older adults is to go beyond chronological age and examine whether a person is experiencing optimal aging (like the gentleman pictured in Figure 8 who is in very good health for his age and continues to have an active, stimulating life), normal aging (in which the changes are similar to most of those of the same age), or impaired aging (referring to someone who has more physical challenge and disease than others of the same age).

VIDEO ALERT!

<https://www.youtube.com/playlist?list=PLLTWYyyYIAAFIcYwnB8m8jrSrZv6QWp0R>

Basic Counselling Skills





Counsellors may help clients with specific problems in the present, but they may also support clients with long-term problems stemming from the past too.

Rapport building is at the root of effective communication. Especially when providing psychosocial support, knowing how to develop rapport is key to the success of the therapeutic process.

When offering psychosocial support, counselling skills come into play as a way of the care giver to create and maintain rapport with the client. Some of the counselling skills include:

Listening and attending: Active listening is a term commonly used in relation to counselling. Several factors that are necessary for complete listening, and these include the observation of clients' non-verbal behaviour, as well as understanding verbal content and meaning. It goes without saying that the 'way' something is said is just as important as the actual words spoken. This last point is especially relevant in the counselling context, for clients often have difficulty in finding exactly the right words to express the way they feel. In these circumstances, accompanying non-verbal cues sometimes speak much more eloquently than words. Listening and attending are, therefore, skills that always go together in counselling. This is because it is not possible for counsellors to give clients their full attention without actively listening to them.

Paraphrasing: The word ‘paraphrasing’ refers to the rewording of the content of what clients say. Reflection and paraphrasing are very similar; the difference between them being that the former is generally used to describe a rewording of the emotional content, while the latter is mainly concerned with the factual.

Summarising: The skill of summarising is used when a helper wishes to respond to a series of statements or, in the case of counselling, to a whole session. Clients often talk at random, and they are frequently side-tracked into other related (and sometimes unrelated) issues. This can make it difficult to monitor everything they say, and formulating an accurate summary also requires active listening and an ability to draw all the random threads together into a more coherent framework.

Asking questions: One of the difficulties you may find in listening to others is that your curiosity is aroused, and you are tempted to ask questions in order to get the detail clear in your own head. Most of us probably ask too many questions in our communications with other people, and active listening is often diminished as a result. Questions can be especially problematic in counselling because counsellors sometimes ask questions in order to get the facts straight for themselves, rather than from a desire to understand the client’s subjective experience of things.

Other skills include:

Encouraging clients to be specific

Reflecting their feelings

Helping them to clarify their thoughts

Encouraging them to focus on key issues

Offering forms of challenge when needed

VIDEO ALERT!

3 Quick Rapport Building Techniques:
<https://www.youtube.com/watch?v=FU4TL4-Rpo8>

Principles of counselling

Several foundational principles are the underpinnings of best ethical practice and are viewed as central to the process of ethical decision making within the helping professions

A description of each of the five foundational principles of counselling is as follows:

Autonomy is the principle that addresses respect for independence, and self-determination. The essence of this principle is allowing an individual the freedom of choice and action. It addresses the responsibility of the counselor to encourage clients, when appropriate, to make their own decisions and to act on their own values

Justice: Justice means not discriminating any clients. It means that psychologists should be in a position to execute reasonable judgment and take precautions to ensure that their potential biases do not lead to any unjust (discriminating) practices. An injustice occurs when some benefit to which a person is entitled is denied without good reason or when some burden is imposed unduly.

Beneficence reflects the counselor's responsibility to contribute to the welfare of the client. Simply stated, it means to do good, to be proactive, and also to prevent harm when possible. Beneficence can come in many forms, such as prevention and early intervention actions that contribute to the betterment of clients.

Non-maleficence is the concept of not causing harm to others. Often explained as "above all, do no harm," this principle is considered by some to be the most critical of all the principles, even though theoretically they are all of equal weight. This principle reflects both the idea of not inflicting intentional harm, and not engaging in actions that risk harming others. Weighing potential harm against potential benefits is important in a counselor's efforts toward ensuring "no harm."

Fidelity involves the notions of loyalty, faithfulness, and honoring commitments. Clients must be able to trust the counselor and have faith in the therapeutic relationship if growth is to occur. Therefore, the counselor must take care not to threaten the therapeutic relationship or to leave obligations unfulfilled.

Identifying patients in need of psycho-social support

It is important for all community health care workers to understand mental illness and learn how to identify the signs and symptoms. Research shows that:

Approximately 20% of adults in the U.S., or 40 3.8 million people, experience a mental illness each year according to the National Alliance on Mental Illness

Nearly one out of every five youths from age 13 to 18 will experience a severe mental disorder during their lifetime

Each year, serious mental illness costs \$193.2 billion in lost earnings

Those who live with a serious mental illness also face an increased risk of chronic health conditions

Over one-third of students with a mental health condition dropout of high school

More than 90% of those who die by suicide have symptoms of a mental health condition.

1% of adults live with schizophrenia

6% with bipolar disorder

9% have experienced at least one major depressive episode

Trying to tell the difference between expected behavior and signs of mental illness is not always easy, especially in people who are under stress. Although there is no easy test and each disorder has its own symptoms, there are some common signs of mental illness to help you identify those who may benefit from psychosocial health support.

Anxiety and depression: We all feel stressed or sad from time to time. However, anxiety and depression can be a sign of mental health issues when it is constant and interferes with other activities. If you have noticed your client has lost interest, seems sad or irritable all the time, and lacks motivation and energy, they may be suffering from more than sadness. Symptoms of anxiety may include shortness of breath, headaches, diarrhea or a racing mind. Your client may not share these symptoms with you, but by watching their behavior and their ability to stay on track during a conversation, the symptoms may become evident.

Extreme mood changes: We all experience times when moods change, but sudden and dramatic changes in mood, such as uncontrollable “highs,” feelings of euphoria, prolonged or strong feelings of anger or irritability may be symptoms of mental illness.

Changes in sleeping habits or feeling constantly fatigued: To maintain optimal health, we need 8 hours of sleep each night. When an individual experiences persistent changes in sleeping patterns, this might indicate a mental illness. Insomnia can be a sign of anxiety, depression or substance abuse. In some cases, your client may be sleeping 8 or more hours and still feel fatigued on a consistent basis. This is another indication they may be struggling with a mental health condition.

Weight or appetite changes: Some people eat more when they are under stress or feeling bad, and others eat less. This effects weight gain or loss and is also a warning sign of a mental health condition or eating disorder.

Inability to deal with daily activities: This may include avoiding friends and social activities, changes in sex drive, or recent withdrawal and loss of Interest in activities your client previously enjoyed. Students may demonstrate an unusual drop in function at school, such as quitting sports or failing grades. Adults may demonstrate a similar drop in function at work, such as not meeting deadlines and poor performance.

Heightened sensitivity: Demonstrating a heightened sensitivity to sights, sounds, smells or touch, such as avoiding over-stimulating situations or jumping dramatically at loud noises, may also indicate mental health conditions.

Feeling guilty or worthless: Some experiencing mental health conditions express feelings or thoughts such as “I’m a failure,” “It’s my fault” or “I’m worthless.”

Difficulty relating to other people: Your client may express confused thinking or constant problems concentrating, difficulty understanding or relating to people and difficulty perceiving reality, including an inability to perceive changes in their own feelings that may appear as a lack of insight

Substance abuse: If it appears your client is drinking an excessive amount of alcohol, relying on alcohol each day, using other recreational or prescription drugs to cope with life or exhibiting drug-seeking behavior, they may suffer from an addiction problem and an underlying mental health condition for which they are attempting to self-medicate.

Multiple physical ailments without obvious causes: Vague physical complaints, such as headaches, stomach aches and ongoing aches and pains without a current diagnosis explaining the symptoms, is another sign of a mental health condition. In this case, it is important your client is evaluated for an underlying medical condition which may not have been diagnosed.

Assessment of the patient for psychological health care needs

Psychological assessment contributes important information to the understanding of individual characteristics and capabilities, through the collection, integration, and interpretation of information about an individual. Such information is obtained through a variety of methods and measures, with relevant sources determined by the specific purposes of the evaluation and these methods and measures are known as the ***assessment tools***.

Key factors that should be considered when selecting an assessment tool include:

the purpose for which the tests are being used

Users Knowledge of typical items, quantity and range of items

relevance of the test items to the job criteria

Potential acceptability to clients

accuracy of the measures (including risks associated with unfair discrimination and test bias)

costs of materials and other implementation costs

suitability of automated reports available to audience

Major assessment tools include:

Psychological test is an objective and standardized measure of an individual's mental and/or behavioral characteristics. Psychological tests include *personality test* (Rorschach inkblot test, Thematic Apperception Test (TAT), Sentence completion test, Create drawings (Draw a person test) or complete a story. Minnesota Multiphasic Personality Inventory, Millon Clinical Multiaxial Inventory (MCMI), The California Psychological Inventory (CPI) , Sixteen Personality Factor Questionnaire (or 16PF) , The Eyesenck Personality Inventory, Myer Briggs Type Indicator (MBTI),

Intelligence tests (Stanford-Binet Intelligence Scales and Wechsler intelligence scales(Wechsler intelligence scale for children and Wechsler Adult Intelligence Scale)

Neuropsychological testing: Luria- Nebraska battery, Halsted- Reitan battery Michigan neuropsychological battery, Shipley Institute of Living Scale, NIMHANS neuropsychological battery.

The Interview: The interview as a tool of psychological assessment typically involves more than talk. If the interview is conducted face-to-face, then the interviewer is probably taking note of not only the content of what is said but also the way it is being said. More specifically, the interviewer is taking note of both verbal and nonverbal behavior. Nonverbal behavior may include the interviewee's "body language," movements and facial expressions in response to the interviewer, the extent of eye contact, and apparent willingness to cooperate.

The Portfolio: Students and professionals in many different fields of endeavor ranging from art to architecture keep files of their work products. These work products—whether retained on paper, canvas, film, video, audio, or some other medium—constitute what is called a *portfolio*.

Case History Data: Case history data refers to records, transcripts, and other accounts in written, pictorial, or other form that preserve archival information, official and informal accounts, and other data and items relevant to an assessee. Case history data may include files or excerpts from files maintained at institutions and agencies such as schools, hospitals, employers, religious institutions, and criminal justice agencies.

Behavioral Observation: If you want to know how someone behaves in a particular situation, observe his or her behavior in that situation. Such "down-home" wisdom underlies at least one approach to evaluation. Behavioral observation, as it is employed by assessment professionals, may be defined as monitoring the actions of others or oneself by visual or electronic means while recording quantitative and/or qualitative information regarding the actions.

Role-Play Tests: Role play may be defined as acting an improvised or partially improvised part in a simulated situation. A role-play test is a tool of assessment wherein assessees are directed to act as if they were in a particular situation. Assesseees may then be evaluated with regard to their expressed thoughts, behaviors, abilities, and other variables.

Assessing patients involves the following components:

General Observations: Appearance, Speech, Behavior, Cooperativeness

Thinking: Thought Process, Thought Content, Perceptions

Emotion: Mood, Affect,

Cognition: Orientation/Attention, Memory, Insight, Judgment

Psychosocial Health Needs

Psychosocial needs is a term used to represent patient and family's mental, social, cultural, spiritual, and developmental needs arising from emotional responses to their diagnosis, social and role limitations, loss of physical and/or mental abilities. These needs may include:

Debilitating disease

Stress

Depression

Anxiety

Grief

Burn - out

Table 1.1 Psychosocial Needs and Formal Services to Address Them

Psychosocial Need	Health Services
Information about illness, treatments, health, and services	Provision of information, e.g., on illness, treatments, effects on health, and psychosocial services, and help to patients/families in understanding and using the information
Help in coping with emotions accompanying illness and treatment	Peer support programs Counseling/psychotherapy to individuals or groups Pharmacological management of mental symptoms
Help in managing illness	Comprehensive illness self-management/self-care programs

Assistance in changing behaviors to minimize impact of disease	Behavioral/health promotion interventions, such as: Provider assessment/monitoring of health behaviors (e.g., smoking, exercise) Brief physician counseling Patient education, e.g., in cancer-related health risks and risk-reduction measures
Material and logistical resources, such as transportation	Provision of resources
Help in managing disruptions in work, school, and family life	Family and caregiver education Assistance with activities of daily living (ADLs), instrumental ADLs, chores Legal protections and services, e.g., under the children's Act Cognitive testing and educational assistance
Financial advice and/or assistance	Financial planning/counseling, including management of day-to-day activities such as bill paying Insurance (e.g., health, disability) counseling Eligibility assessment/counseling for other benefits (e.g., Supplemental Security Income, Social Security Disability Income) Supplement financial grants

Components of psychosocial healthcare

Psychosocial health is the result of complex interaction between a person's history and his or her thoughts about and interpretations of the past and what the past means to

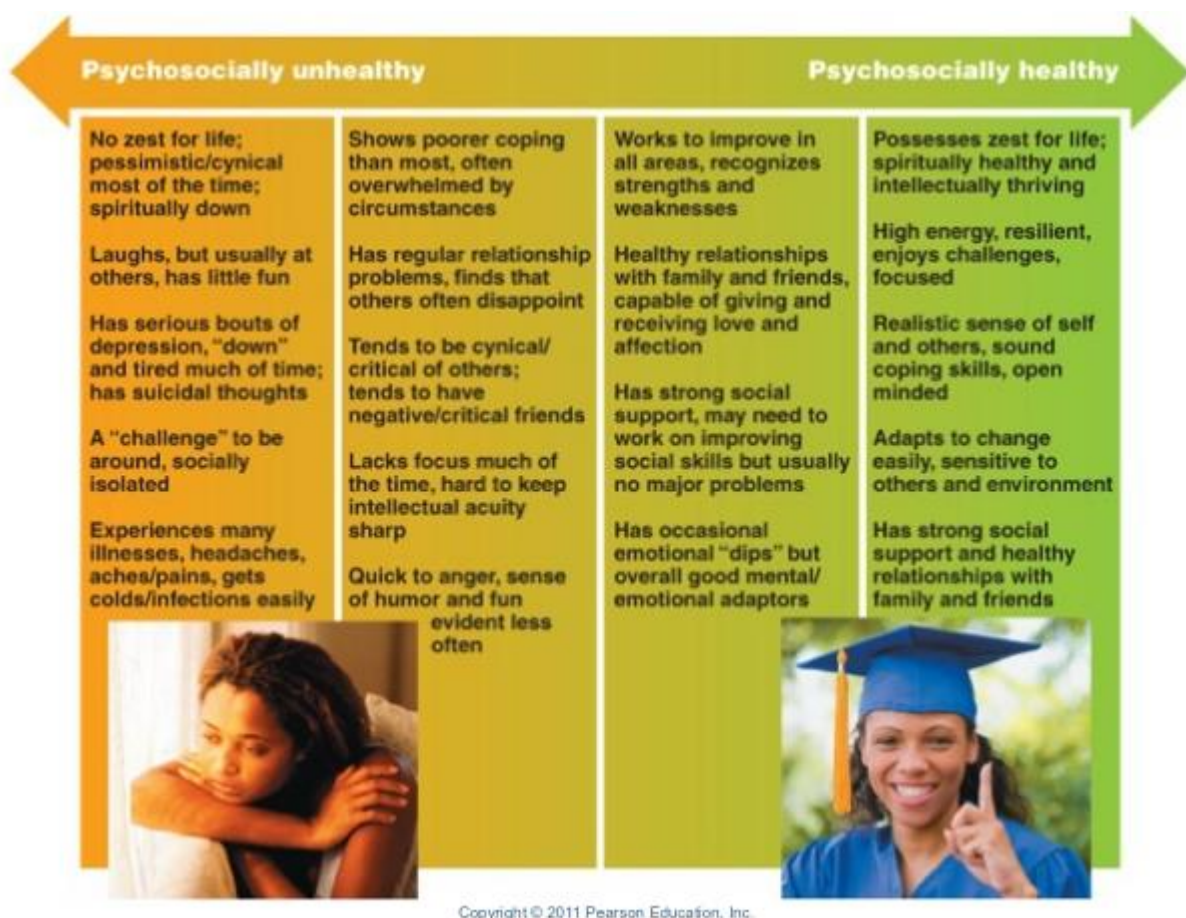
The major components of Psychosocial health are:

Mental health: The thinking you. This is the "thinking" or "rational" part of psychosocial health. Mentally healthy people tend to respond to life's challenges constructively. Irrational thinking may indicate poor mental health.

Emotional Health: The feeling you. The “feeling” or subjective side of psychosocial health that includes emotional reactions to life. Emotions are intensified feelings and complex patterns and include: love, hate, frustration, anxiety, and joy.

Social health: Interactions with others. This is the aspect of psychosocial health that includes interactions with others, ability to use social supports, and ability to adapt to various situations. It also includes social bonds and social support.

Spiritual health: An inner quest for well-being. This refers to a belief in a unifying force that gives purpose or meaning to life, a sense of belonging, a feeling of connection to other people, to nature, or God or other higher power.



Psychosocial health care may include the following aspects:

Basic counseling

Drug adherence

Feeding

Advise for medical checkup

Accompanying for medical care

Learning Activities

7.1.4.1 Practical Activity

Task 1: Attend the nearest GBV center in your community and carry out a psychosocial assessment of the health care workers in that community.

7.1.4.2 Self-Assessment

You are provided with the following questions for self -assessment, attempt them and check your responses

What is human psychology?

Who is a developmentalist?

Which are the major components of psychosocial health?

What are the 8 stages of lifespan development?

Which personality tests can you use to assess a client for psychosocial needs?

Tools, Equipment, Supplies and Materials

The following resources are provided:

A counseling room with seats, a desk and tissues

Assessment documents

Computers

Stationery

Note book

7.1.5 References

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Responses to self - assessment questions

What is human psychology?

Human Psychology refers the scientific study of behavior and mental processes.

Who is a developmentalist?

It refers to any scholar—regardless of discipline—who seeks to understand the developmental process

Which are the major components of psychosocial health?

Mental health

Emotional Health

Social health

Spiritual health

What are the 8 stages of lifespan development?

Prenatal development

Infancy/toddlerhood

Early childhood

Middle childhood

Adolescence

Young adulthood

Middle adulthood

Late adulthood

Which personality tests can you use to assess a client for psychosocial needs?

Rorschach inkblot test, Thematic Apperception Test (TAT), Sentence completion test, Create drawings (Draw a person test) or complete a story. Minnesota Multiphasic Personality Inventory, Millon Clinical Multiaxial Inventory (MCMI), The California Psychological Inventory (CPI) , Sixteen Personality Factor Questionnaire (or 16PF) , The Eyesenck Personality Inventory, Myer Briggs Type Indicator (MBTI),

Learning Outcome 2: Care for patients with psychosocial health needs

7.2.1 Introduction to the learning outcome

This learning outcome specifies the content of competencies required to care for patients with psychosocial health need. It includes prioritizing psychological needs, standards for psychosocial health care services, psychosocial healthcare team, collaborative approaches, effective communication in psychosocial health care, critical thinking and Care for the care giver

7.2.2 Performance Standard

2.1 Psychosocial health needs are prioritised as per the assessment report analysis.

2.2 Specific psychosocial health care is given as per Standards for Psychosocial Healthcare and WHO standards.

2.3 Requirements for offering psychosocial health care support are identified and gathered as per WHO standards and work place policy.

2.4 Collaborative care between psychosocial and biomedical care is provided as per work policy, Standards for Psychosocial Healthcare and WHO standards

2.5 Patient is linked to psychosocial services as per work policy, Standards for Psychosocial Healthcare and WHO standards

7.2.3 Information Sheet

Definition of terms

Mental health: This refers to our emotional, psychological, and social well-being. It affects how we think, feel, and act. It also helps determine how we handle stress, relate to others, and make choices.

Case Management: This is a collaborative process of assessment, planning, facilitation, care coordination, evaluation and advocacy for options and services to meet an individual's and family's comprehensive health needs through communication and available resources to promote patient safety, quality of care, and cost effective outcomes.

Screening: This is the preliminary assessment of a client to ascertain his or her fitness for health related or psychiatric therapy in general, a particular treatment strategy,

Prioritizing psychological needs

The World Health Organization (WHO) embraces a definition of health as “physical, mental, and social well-being”. Of these elements, mental well-being historically has been misunderstood and often forgotten. WHO has spent the last five years actively addressing the barriers that prevent access to mental health care and campaigning for the full incorporation of mental health in worldwide public health.

Mental health is described by WHO as:

“... a state of well-being in which the individual realizes his or her own abilities, can cope with the normal stresses of life, can work productively and fruitfully, and is able to make a contribution to his or her community.”

In this positive sense mental health is the foundation for well-being and effective functioning for an individual and for a community. This core concept of mental health is consistent with its wide and varied interpretation across cultures.

The *Theory of Basic Psychological Needs* postulates that human beings have three inborn basic needs: autonomy, competence, and relatedness.

The *need for autonomy* is the freedom to determine, plan and implement one's behaviours and living- as different from being independent. The system of selfhood that is related with the system of autonomy, and social supports are the primary determinants of how to cope with stress

Competence is one's belief in his or her ability to cope with a certain state. People need a certain amount of effectiveness and confidence in their activities. Negative feedbacks given by others will reduce the feeling of competence and may discourage individuals and cause them not to take part in activities.

Relatedness is the need for belonging. It enables individuals to interact with a warm and friendly atmosphere and with people providing such an environment, and to experience the

sense of belonging and relatedness. An individual lives the self in such an environment. It is thought that environments supporting autonomy require more activeness and more participation, which facilitate individuals' development. People need to contact and interact with others and to feel that the relations exist and will continue to exist.

Psychological health depends on satisfying the three needs. All of the needs contribute to effective functioning and to psychological health. None of them could be disclaimed. Besides, needs are complementary to each other rather than in conflict. The three basic needs are the unchanging universal tendencies which are inherently available in humans and which therefore do not differ from culture to culture. Meeting the needs, which is a nutrient to human soul, feeds the psychological well being while failure to meet the needs or hindering it, that is to say, hunger due to the lack of food, may cause ill being.

Standards for psychosocial health care services

Many different providers of health services—some in oncology, some delivering health care for other complex health conditions—recognize that psychosocial problems can have both direct and indirect effects on health and have developed interventions to address them. Some of these interventions are derived from theoretical or conceptual frameworks; some are based on research findings; and some have undergone empirical testing.

The best have all three characteristics. When viewed together, these interventions evidence common elements that point to a model for the effective delivery of psychosocial health services. The components of this model include;

identifying patients with psychosocial health needs that are likely to affect their health or health care, and developing with patients appropriate plans for

linking patients to appropriate psychosocial health services,

supporting patients in managing their illness,

coordinating psychosocial with biomedical health care, and

following up on care delivery to monitor the effectiveness of services and determine whether any changes are needed.

Effective patient–provider communication is central to all of these components. A combination of several activities rather than any single activity by itself (e.g., screening, case management, illness self-management) is needed to deliver appropriate psychosocial health care effectively to individuals with complex health conditions. This conclusion also is supported by the findings of several systematic reviews of psychosocial care. For example, not surprisingly, screening by itself is less effective than screening with follow-up. The U.S. Preventive Services Task Force, for instance, recommends screening for depression in adults in clinical practices only when practices have systems in place to ensure effective follow-up

treatment and ongoing monitoring. This recommendation reflects research finding that only small benefits result from screening by itself, but larger benefits when screening is accompanied by effective follow-up.

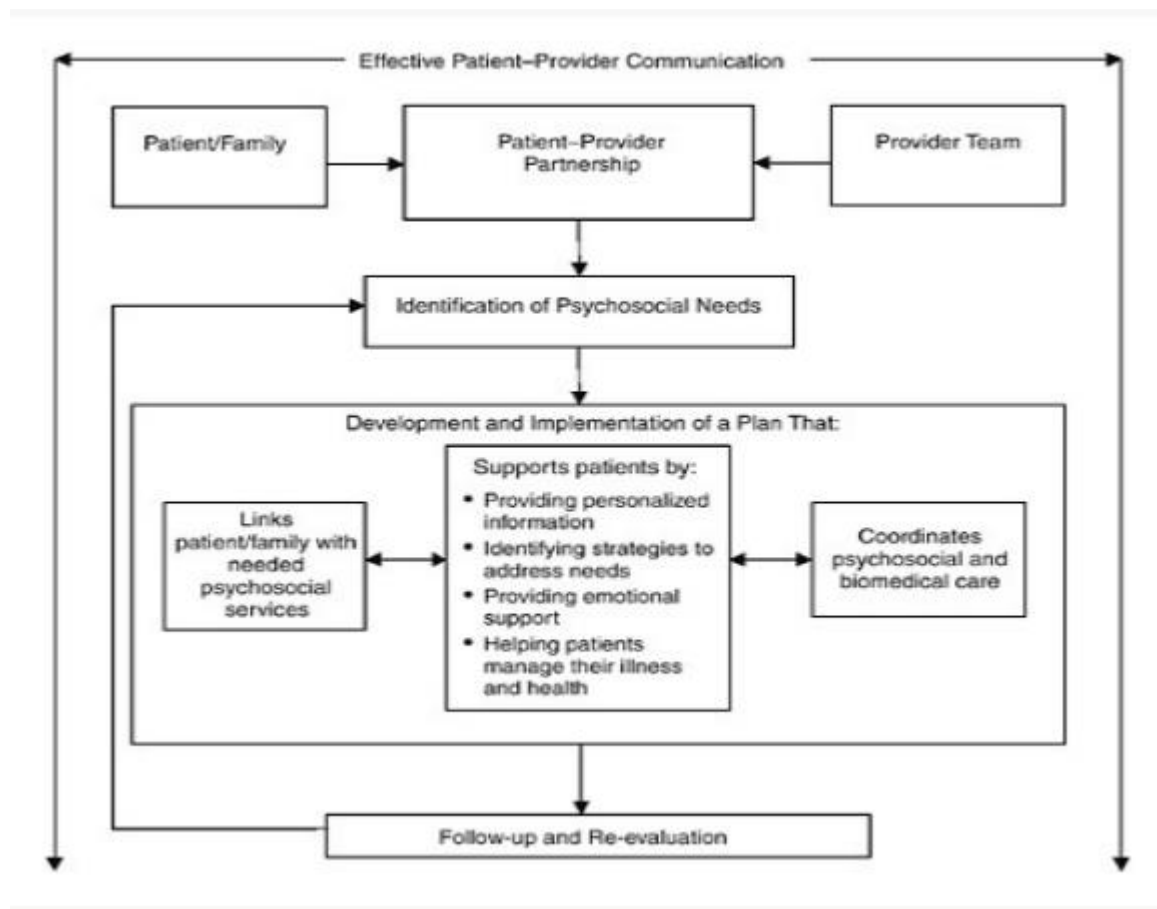


Figure 1.1 Model for the delivery of psychosocial health services.

Collaborative approaches

This is also known as care coordination. Care coordination is the deliberate organization of patient care activities between two or more participants (including the patient) involved in a patient's care to facilitate the appropriate delivery of health care services. Organizing care involves the marshalling of personnel and other resources needed to carry out all required patient care activities, and is often managed by the exchange of information among participants responsible for different aspects of care. Care coordination as a psychosocial intervention must fulfill a dual function: coordination of psychosocial health services with biomedical services, and coordination of biomedical care provided by multiple clinicians.

Healthcare systems around the world face both increasing demands and inequality in service distribution. The current trend is for collaboration among healthcare actors, named as collaborative healthcare, in order to address challenges such as these to improve the social sustainability of the system. That is to provide accessible and equitable healthcare services to meet people's health and well-being needs.

In order to address the complex social, emotional, psychological, and spiritual needs patients, health care workers collaborate with a number of psychosocial colleagues, including but not limited to:

social workers,

child life specialists,

chaplains,

educators,

creative arts therapists,

Clinical psychologist,

Counselor, and

Medical clinicians

Although psychosocial teams differ across institutions in size and composition, they all work to integrate their individual expertise and provide the highest quality care for patients and families.

Essential elements of successful psychosocial collaboration include

shared goals,

role clarity, and

effective communication.

Effective communication in psychosocial health care

At the heart of an effective model of psychosocial care delivery is a well-functioning patient–provider partnership, characterized, in large part, by effective communication. Communicating effectively means that patients are able to receive and understand information about their illness and health care, and clearly express their needs for assistance and the values and personal resources that will shape the health care system’s response to these needs.

Patients should be comfortable with asking questions of all their care providers and equally comfortable with responding to questions posed to them. They should be competent and at ease as a member of their own health care team, which will make decisions about the best

strategy for addressing their illness. Patients with language barriers, cognitive deficits, or other impediments to communication should receive assistance in overcoming these barriers to effective communication.

In many instances, members of the patient's family also are involved in this communication. In pediatric cases, a family member may be the primary communicator and participant in planning care; with adults, some patients may also have limited capacity to communicate. Even when adult patients are able to communicate, family members are key caregivers, especially for older adults. Treatment planning and planning for managing the effects of illness requires communication with the patient's caregivers as well as with the patient. When patients do not have the capacity to participate actively themselves, there needs to be an explicit substitute who is legally and psychologically able to act as the patient's advocate.

Care providers similarly should possess the communication skills necessary to be effective clinicians and supportive partners in care—a hallmark of high-quality health care and health care professionals. These communication skills include establishing a good interpersonal relationship with the patient.

Key aspects of effective patient–clinician communication identified in the National Cancer Institute (NCI) report *Patient-Centered Communication in Cancer Care* include:

fostering healing relationships,

exchanging information,

responding to emotions,

managing uncertainty,

making decisions, and

enabling patient self management

Linking Patients to Psychosocial Health Services

Several mechanisms used to link patients with psychosocial health services delivered by health and human service providers have empirical support, although the strength of this support varies. These mechanisms include structured referral arrangements and formal agreements with external providers, case management, and collocation and clinical integration of services. Use of care/system navigators is also being studied for its effectiveness in linking patients with needed services.

Structured Referral: Although referral to other organizational or individual providers is a common mechanism for linking individuals with psychosocial health services, there has been little study of the general effectiveness of such referrals. Most studies of referral have

addressed referrals between physicians. These and one Australian study of referrals of cancer patients to psychosocial services indicate high rates of failure to connect individuals to the referred providers, frequent failure of the referred individuals to accept the referred services, and failure to track the outcomes of referrals. Referring patients to external providers is likely to continue to be a primary mechanism for linking patients with needed psychosocial health services because it requires fewer organizational and physical plant resources than offering all psychosocial services on site. To conserve both psychosocial health services and the personnel and resources needed to make referrals, however, it is important for providers to implement approaches for doing so efficiently and effectively.

Case Management: Case management consists of a variety of activities necessary to coordinate some or all of the health-related care needed by patients. Although these activities often vary with the severity of the illness, the needs of the individual, and the specific model of case management employed, case management services that address psychosocial health needs typically include assessment of the patient's need for supportive services; individual care planning, referral, and connection of the patient with other necessary services and supports; ongoing monitoring of the patient's care plan; advocacy and education; and monitoring of the patient's symptoms. These activities can be performed by an appointed individual or a group.

On-site Collocation and Clinical Integration of Services: Multiple studies of mental health care have found that same-site delivery of mental health and primary care is effective in linking patients to the collocated services and can improve treatment outcomes. In a 1995 study of a nationally representative sample of outpatient drug use treatment units, same-site delivery of services was more effective than formal arrangements with external providers, referral agreements, or case management in ensuring that patients would utilize necessary services (a first step in collaborative care). Integrating psychosocial health care into medical care settings facilitates patient follow-through on referrals, allows for face-to-face verbal communication in addition to or as an alternative to communicating in writing, and allows for informal sharing of the views of different disciplines and easy exchange of expertise. Studies of care collaboration also have shown that physical proximity of would-be collaborators facilitates collaboration.

Care/System Navigators: Use of care/system navigators, as well as individual patient advocates, is similar to case management and may also help link patients to needed psychosocial services. Such programs in cancer care were developed initially to help low-income patients participate in screening for the detection of cancer and aid those with suspicious screening findings in gaining access to diagnostic and treatment services. Initially, patient navigators tended to be local community residents without professional credentials, but more recently some have proposed that nurses, social workers, and other health workers play a navigator role. This variation in the background of the navigator relates to differences in role expectations. While all navigator programs focus on helping patients overcome barriers to receiving effective services, some also include patient education and patient advocacy roles.

Critical Thinking

Critical thinking is the analysis of an issue or situation and the facts, data or evidence related to it. Ideally, critical thinking is to be done objectively—meaning without influence from personal feelings, opinions or biases—and it focuses solely on factual information.

Critical thinking is a skill that allows you to make logical and informed decisions to the best of your ability. For example, a child who has not yet developed such skills might believe the Tooth Fairy left money under their pillow based on stories their parents told them. A critical thinker, however, can quickly conclude that the existence of such a thing is probably unlikely—even if there are a few bucks under their pillow.

1. Identification

The first step in the critical thinking process is to identify the situation or problem as well as the factors that may influence it. Once you have a clear picture of the situation and the people, groups or factors that may be influenced, you can then begin to dive deeper into an issue and its potential solutions.

How to improve: When facing any new situation, question or scenario, stop to take a mental inventory of the state of affairs and ask the following questions:

Who is doing what?

What seems to be the reason for this happening?

What are the end results, and how could they change?

2. Research

When comparing arguments about an issue, independent research ability is key. Arguments are meant to be persuasive—that means the facts and figures presented in their favor might be lacking in context or come from questionable sources. The best way to combat this is independent verification; find the source of the information and evaluate.

How to improve: It can be helpful to develop an eye for unsourced claims. Does the person posing the argument offer where they got this information from? If you ask or try to find it

yourself and there's no clear answer, that should be considered a red flag. It's also important to know that not all sources are equally valid—take the time to learn the difference between popular and scholarly articles.

3. Identifying biases

This skill can be exceedingly difficult, as even the smartest among us can fail to recognize biases. Strong critical thinkers do their best to evaluate information objectively. Think of yourself as a judge in that you want to evaluate the claims of both sides of an argument, but you'll also need to keep in mind the biases each side may possess.

How to improve: Challenge yourself to identify the evidence that forms your beliefs, and assess whether or not your sources are credible.

First and foremost, you must be aware that bias exists. When evaluating information or an argument, ask yourself the following:

Who does this benefit?

Does the source of this information appear to have an agenda?

Is the source overlooking, ignoring or leaving out information that doesn't support its beliefs or claims?

Is this source using unnecessary language to sway an audience's perception of a fact?

4. Inference

The ability to infer and draw conclusions based on the information presented to you is another important skill for mastering critical thinking. Information doesn't always come with a summary that spells out what it means. You'll often need to assess the information given and draw conclusions based upon raw data.

The ability to infer allows you to extrapolate and discover potential outcomes when assessing a scenario. It is also important to note that not all inferences will be correct. For example, if you read that someone weighs 260 pounds, you might infer they are overweight or unhealthy. Other data points like height and body composition, however, may alter that conclusion.

How to improve: An inference is an educated guess, and your ability to infer correctly can be polished by making a conscious effort to gather as much information as possible before jumping to conclusions. When faced with a new scenario or situation to evaluate, first try skimming for clues—things like headlines, images and prominently featured statistics—and then make a point to ask yourself what you think is going on.

5. Determining relevance

One of the most challenging parts of thinking critically during a challenging scenario is figuring out what information is the most important for your consideration. In many scenarios, you'll be presented with information that may seem important, but it may pan out to be only a minor data point to consider.

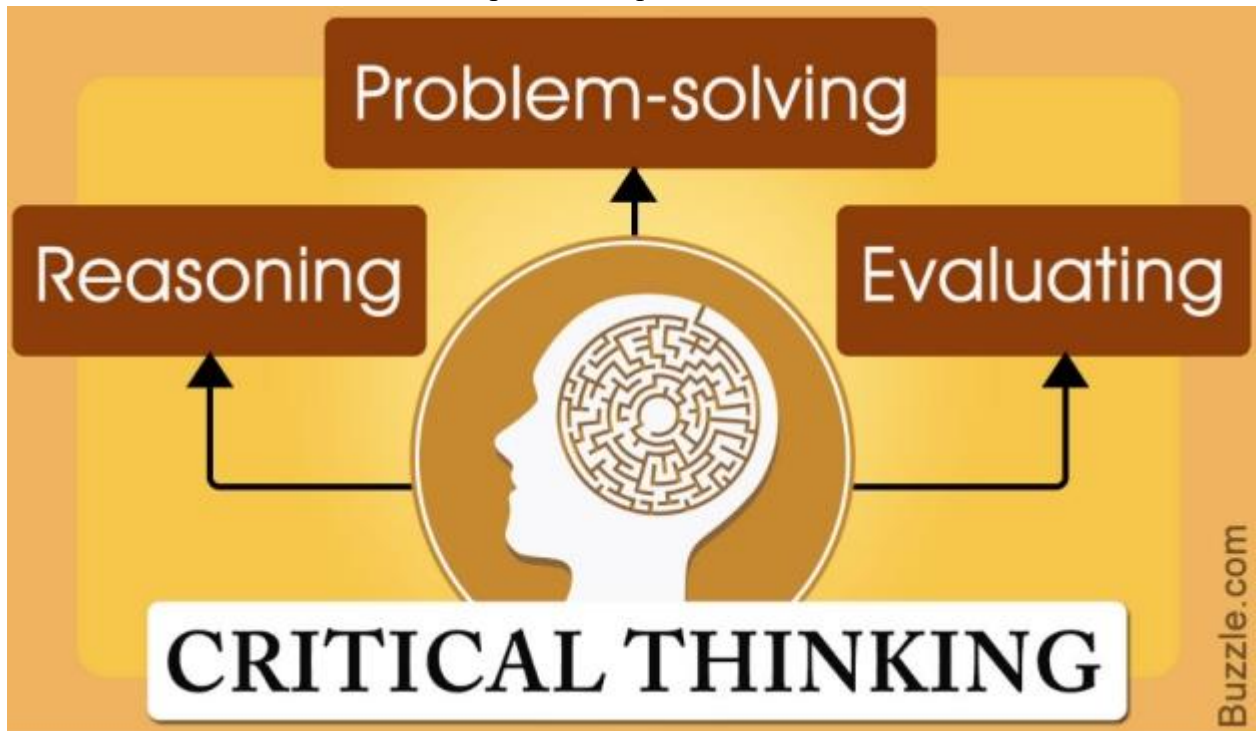
How to improve: The best way to get better at determining relevance is by establishing a clear direction in what you're trying to figure out. Are you tasked with finding a solution? Should you be identifying a trend? If you figure out your end goal, you can use this to inform your judgement of what is relevant.

Even with a clear objective, however, it can still be difficult to determine what information is truly relevant. One strategy for combating this is to make a physical list of data points ranked in order of relevance. When you parse it out this way, you'll likely end up with a list that includes a couple of obviously relevant pieces of information at the top of your list, in addition to some points at the bottom that you can likely disregard. From there, you can narrow your focus on the less clear-cut topics that reside in the middle of your list for further evaluation.

6. Curiosity

It's incredibly easy to sit back and take everything presented to you at face value, but that can also be also a recipe for disaster when faced with a scenario that requires critical thinking. It's true that we're all naturally curious—just ask any parent who has faced an onslaught of “Why?” questions from their child. As we get older, it can be easier to get in the habit of keeping that impulse to ask questions at bay. But that's not a winning approach for critical thinking.

How to improve: While it might seem like a curious mind is just something you're born with, you can still train yourself to foster that curiosity productively. All it takes is a conscious effort to ask open-ended questions about the things you see in your everyday life, and you can then invest the time to follow up on these questions.



Caring for the caregiver

Caregiving can be physically and emotionally exhausting. Whether you are in the profession of caregiving or taking care of a loved one, it is important to remember to recharge your batteries. For family members, caregiving can also lead to additional pressures, such as financial strain, family conflict, and social withdrawal. Over time, caregiver stress can lead to burnout, a condition marked by irritability, fatigue, problems with sleep, weight gain, feelings of helplessness or hopelessness, and social isolation.

Caregiver burnout is an example of how repeated exposure to stress harms mental and physical health. Chronic stress triggers a release of stress hormones in the body, which can lead to exhaustion, irritability, a weakened immune system, digestive distress, headaches, pains, and weight gain, especially in the midsection of the body.

3 ways to care for yourself if you are a caregiver

Self-compassion is essential to self-care: Being kind to yourself builds the foundation to self-care. Self-compassion means giving yourself credit for the tough, complex work of

caregiving, stepping away from the self-critical, harsh inner voice, and allowing yourself time — even if it's just a few minutes a day — to take care of yourself. Lack of time or energy can make getting that time away particularly challenging. You may even feel guilty or selfish for paying attention to your own needs. What you need to know is this: in fact, practicing self-care allows the caregiver to remain more balanced, focused, and effective, which helps everyone involved.

Make eating well and getting quality sleep priorities: It's easy to forget about your own meals and needs when trying to help others. Maintaining adequate sleep and nutrition are key to preventing caregiver burnout. Build a daily 10-minute nighttime routine to achieve more restful sleep. Missing meals can lead to irritability and fatigue, so it is important to eat regularly scheduled meals throughout the day. Nutrition can also be an important factor to prevent burnout. Chronic stress has been linked to increased inflammation in the body, so it is helpful to avoid foods that are processed or high in refined sugars, which increase inflammation in the body. Avoid or reduce alcohol, since alcohol both increases inflammation in the body and disrupts quality of sleep.

Remain socially connected. Find support through local caregiver support groups: While it can be difficult to keep social appointments with friends and family in the face of medical caretaking, it is important to maintain social connections to feel less isolated and prevent burnout. Realizing that you're not alone and that others are going through similar experiences nurtures your ability to be self-compassionate. Hospitals and local organizations often offer caregiver support groups for family and caregivers.

7.2.4 Learning Activities

Role Play activities

Task 1: Mrs. Maliwaza is 34 years old, 6 months pregnant and has visited the health center 2 times since for prenatal clinic. As you conduct a home visit to check out on the patients at the health center, Mrs. Maliwaza explains that she has been feeling dizzy, fatigued and has been vomiting all the time she eats.

In groups of two, role play how you would link her with relevant health care providers.

Self-Assessment

You are provided with the following questions for self -assessment, attempt them and check your responses

What are the 3 psychosocial needs as postulated by the psychological needs theory?

Which psychosocial support workers can a health care worker collaborate with to effectively meet patients' psychosocial needs?

What mechanisms can you use to link a patient with other related service?

In the provision of psychosocial health care, why is important to have an efficient client - clinician communication?

What are the essential elements of successful psychosocial collaboration?

Tools, Equipment, Supplies and Materials

The following resources are provided:

A counseling room with seats, a desk and tissues

Assessment documents

Computers

Stationery

Note book

References

https://www.who.int/mental_health/evidence/MH_Promotion_Book.pdf

<https://www.nap.edu/read/11993/chapter/6#160>

<https://www.ncbi.nlm.nih.gov/books/NBK4023/>

Responses to self assessment

1. What are the 3 psychosocial needs as postulated by the psychological needs theory?

- Autonomy
- Relatedness
- Competence

2. Which psychosocial support workers can a health care worker collaborate with to effectively meet patients' psychosocial needs?

- social workers,

- child life specialists,
- chaplains,
- educators,
- creative arts therapists,
- Clinical psychologist,
- Counselor, and
- Medical clinicians

3. What mechanisms can you use to link a patient with other related service?

Structured referral arrangements and formal agreements with external providers, case management, and collocation and clinical integration of services and care/system navigators

4. In the provision of psychosocial health care, why is important to have an efficient client - clinician communication?

- fostering healing relationships,
- exchanging information,
- responding to emotions,
- managing uncertainty,
- making decisions, and
- enabling patient self management

5. What are the essential elements of successful psychosocial collaboration?

- shared goals,
- role clarity, and
- effective communication.

Learning Outcome 3: Follow up on psychosocial healthcare support needs

7.3.1 Introduction to the learning outcome

This learning outcome specifies the content of competencies required to follow up on psychosocial health care needs. It includes social support systems, Interpersonal skills, monitoring psychosocial patient's progress, reporting of psychosocial issues

7.3.2 Performance Standard

- I. Patient condition is monitored in accordance with Standards for Psychosocial Healthcare and WHO standards.
- II. **Routine patient care** is provided as per Standards for Psychosocial Healthcare and WHO standards.
- III. **Patient's condition** is reported to **relevant personnel** as per work policy, Standards for Psychosocial Healthcare and WHO standards.
- IV. Faults are reported and rectified as per workplace policy.

7.3.3 Information Sheet

Definition of terms

Social support: support accessible to an individual through social ties to other individuals, groups, and the larger community.

Social support systems

The National Cancer Institute's Dictionary of Cancer Terms defines **social support** as “a network of family, friends, neighbors, and community members that is available in times of need to give psychological, physical, and financial help”.

Theoretical models of social support specify the following two important dimensions:

a structural dimension, which includes network size and frequency of social interactions, and

a functional dimension with emotional (such as receiving love and empathy) and instrumental (practical help such as gifts of money or assistance with child care) components.

Most research has found that quality of relationships (functional dimension) is a better predictor of good health than quantity of relationships (structural dimension), although both are important.

It should be noted that the optimal source of social support may depend on the developmental stage of the person who is receiving the support. For example, parental support seems to be more valuable in early adolescence than it is in late adolescence. It has been shown that the perception of social support is associated with the degree of social interaction in the elderly

and with instrumental support in younger adults. In a sample of childhood sexual abuse survivors, a combination of self-esteem support (the individual perceives that he or she is valued by others) and appraisal support (the individual perceives that he or she is capable of getting advice when coping with difficulties) was most useful in preventing the development of PTSD

The persons, agencies and organizations with which a caregiver has contact – directly or indirectly – are referred to as a person's ***social support system***.

The benefits of having and using personal support systems include reduced stress, decreased physical health problems, and improved emotional well-being. Taking stock of your social support system is an excellent way to assess where your help actually comes from and who provides it. Taking stock can also help you see reasons why you don't ask for or accept assistance, and reasons why people may not offer assistance.

Who are the people and organizations who help you?

Spouse

Parents

Children

Other Relatives

Friends

Other caregivers

Doctor(s)

Nurse(s)

Support group

Church members

Civic group/clubs

Schools

Professional agencies

Public health

Social services

Respite care

Activity programs

Interpersonal skills

Interpersonal skills are measures of how adept you are at interacting with others. Active listening is an interpersonal skill, as is knowing how to communicate to someone else that you respect him or her. When problems arise you use your interpersonal skills to resolve conflict with others. People learn interpersonal skills by interacting with family members, going to school, and socializing with their peers. Healthy interpersonal skills reduce stress, resolve conflict, improve communication, enhance intimacy, increase understanding, and promote joy.

Interpersonal skills include:

Communication skills involve both listening and speaking effectively.

Assertiveness skills involve expressing yourself and your rights without violating others' rights.

Conflict is natural and inevitable. Conflict resolution skills help you resolve differences so that you may continue a relationship effectively.

Anger management skills involve recognizing and expressing anger appropriately in order to achieve goals, handle emergencies, solve problems and even protect our health.

Interpersonal Skills Self-Assessment

Ask yourself the questions below. If you identify with a number of these statements, you would likely benefit from work on your interpersonal skills.

How often do you.....

Seek approval and affirmation from others, but fear criticism?

Guess at what normal behavior is?

Feel as if you are different from other people?

Isolate yourself from and feel afraid of people in authority roles?

Downplay your own accomplishments and good deeds?

Have difficulty following a project through from beginning to end?

Get frightened or stressed in the company of an angry person.
Lie in order to avoid conflict?

Judge yourself harshly?

Feel that others (or society) take advantage of you?

Take yourself seriously, and view other relationships just as seriously?

Have problems developing and maintaining intimate relationships?

Feel guilty when you stand up for yourself or put your needs first?

Feel responsible for others and find it easier to have concern for others than for yourself?

Act impulsive, before considering alternative actions or possible consequences?

Have difficulty feeling or expressing your own feelings?

Monitoring psychosocial patient's progress

Patient follow-up has been cited in identifying effective interventions for improving health care processes and outcomes. Follow-up can take place in a variety of ways, including telephone calls to patients to monitor their status, home visits by care managers or other personnel, as part of a scheduled outpatient visit, or through Internet or web-based technology.

Follow-up involves two discrete activities. First is the determination of which services, if any, the patient used, any problems encountered, and satisfaction with services provided. Second is rescreening and assessment to identify new or unmet needs that are then addressed.

Routine patient care may include but not limited to:

Communication

Social care support

Feeding

Dressing

Washing/tidying up

Visiting

7.3.4 Learning Activities

7.3.4.1 Role Play activities

Task 1: Mrs. Maliwaza is 34 years old, 8 months pregnant. From a previous home visit you linked her with a clinician and she tested HIV positive. She was in denial and was refusing to take her ARVs

In groups of two, role play how you would attach Mrs. Maliwaza to a social support system.

7.3.4.2 Self-Assessment

You are provided with the following questions for self -assessment, attempt them and check your responses

What are a person's social support systems?

Who are the people and organizations who can help in offering psychosocial support to patients?

Which are some of the routine patient care techniques you can use during follow up?

7.3.4.3 Tools, Equipment, Supplies and Materials

The following resources are provided:

A counseling room with seats, a desk and tissues

Assessment documents

Computers

Stationery

Note book

7.3.5 References

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<https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0157474>

Responses to the self assessment

What are a person's social support systems?

The persons, agencies and organizations with which a caregiver has contact – directly or indirectly

Who are the people and organizations who can help in offering psychosocial support to patients?

Spouse, Parents, Children, Other Relatives, Friends, Other caregivers, Doctor(s), Nurse(s), Support group, Church members, Civic group/clubs, Schools, Professional agencies, Public health, Social services, Respite care, Activity programs

Which are some of the routine patient care techniques you can use during follow up?

Communication, Social care support, Feeding, Dressing, Washing/tidying up, Visiting

Learning Outcome 4: Document psychosocial healthcare support

7.4.1 Introduction to the learning outcome

This learning outcome specifies the content of competencies required to document psychosocial health care support. It includes documentation of psychosocial healthcare services, report writing and dissemination, patient agreement, transfer reports and waste disposal

7.4.2 Performance Standard

Patient details are documented in accordance with workplace policy.

Patient's progress reports are prepared and disseminated to relevant authorities as per workplace policy and WHO standards.

Waste is disposed in due regard to environment protection regulations.

7.4.3 Information Sheet

Documentation of psychosocial healthcare services

Health care workers create, and to the extent the records are under their control, maintain, disseminate, store, retain, and dispose of records and data relating to their professional and scientific work in order to:

facilitate provision of services later by them or by other professionals,

allow for replication of research design and analyses,

meet institutional requirements,

ensure accuracy of billing and payments, and

ensure compliance with law.

In provision of psychosocial healthcare services, the main goal for documenting is to provide a complete, accurate recording for follow-up, early detection of complications, and helps to identify areas for improvement.

It also contributes to the circulation of information amongst the different teams involved in the patient's treatment or care.

Appropriate records can also help protect both the client and the psychologist in the event of legal or ethical proceedings.

The medical record is not just a facility document it is also read by Outside readers who may include:

Insurance companies ,

Surveyors ,

Ombudsmen,

Consultants,

Medicare intermediaries,

Authorized family members,

Lawyers

Documentation must include reports of patient and/or family impact of illness, of assessments conducted, nature and impact of psychosocial services provided, monitoring of illness status, and treatment plans.

Psychosocial providers must be familiar with professional codes of ethics and state regulations about protection of the privacy of health information and limits of privacy protection.

Documentation and communication with other professionals must reflect appropriate understanding of what information is essential for team members and/or family members to know, when specific permission is needed, and what different rules apply under extraordinary conditions of risk of harm.

Treatment facilities may impose their own requirements for documentation as long as they comply with state regulations, which represent the final authority. The creation and handling of electronic medical record information should be governed by the same professional rules as other health care communication.

Preparing and disseminating the patient's progress reports

The patient's report must rest on empirically solid data, synthesized and explained at a level that a layman can understand, and answer the central question at hand eg “What’s underlying the patient’s problems with attention?”

To write a good report you should:

Verify that your report relies on solid data. Before the report writing even begins, make sure that the tests you will be citing in the report are backed by the latest research and were used appropriately. That process includes keeping up with the literature regarding whether a test produces different results for different racial/ethnic groups.

Not hide behind the test results: Don’t hide behind the data, but rather write about what those findings reflect about the patient. The main concern, is understanding how to solve the problems that prompted the evaluation

Consider incorporating diversity and cultural context: A report might detail the patient’s family background, romantic relationships and educational or developmental history. But should also include a section that explores other formative experiences, including those that involve race, sexuality or socioeconomic status

Synthesize and conceptualize the findings: Integrate findings from multiple sources into a single section. Our recommendations are much more likely to be taken if they are understood in a very coherent, narrative way how we are conceptualizing them.

Address discordant results: It’s not uncommon for different tests to produce divergent or discordant results. When writing your report, resist the temptation to play up the test you favor and downplay the one with divergent results.

Strip out the jargon: Because numerous individuals may read the report, look at each paragraph and consider: Will a layman understand what I’m saying? If it’s meaningful and palatable to a non-trained reader, it will be for a trained reader as well.

Consider the patient’s perspective: Along with outlining a patient's vulnerabilities, it’s also important—for patients, who will likely read the report, as well as for treatment planning.

Cull the report to its essence: Most of the time when you get a really long report, it’s because someone was doing a data dump rather than an integrated, well-thought-out, thematically organized report. It’s helpful to write a summary of the referral question and primary

conclusion at the beginning of the report and follow with a more detailed explanation further on.

Don't lose sight of the narrative: Check that the report hasn't simply broken down an individual into pieces based on their scores on tests. A report should tell a narrative about the person, and how all these pieces fit together.

While disseminating progress reports on psychosocial health care services, a health care worker ought to:

maintain confidentiality in creating, storing, accessing, transferring, and disposing of records under their control, whether these are written, automated, or in any other medium.

If confidential information concerning recipients of psychosocial services is entered into databases or systems of records available to persons whose access has not been consented to by the recipient, healthcare workers use coding or other techniques to avoid the inclusion of personal identifiers.

make plans in advance to facilitate the appropriate transfer and to protect the confidentiality of records and data in the event of the HCW's withdrawal from positions or practice.

Waste disposal

Sound health care waste management ensures safety and minimal effects on health care workers, the population and the environment.

A comprehensive system should include:

Waste generation and minimization

Separation and segregation of sources

Identification and classification of wastes

Handling and storage

Packaging and labelling

Transport on site and off site

Disposal of residues (including emissions)

Consideration of occupational, public and environmental health and safety

Stakeholder and community awareness and education

Research and development of improved technologies and environmentally friendly practices.

There are various types of Waste that can be generated in offering psychosocial healthcare services. These include:

General waste – non hazardous, poses no risk of injury or infection

Medical waste – material generated in a diagnosis, treatment, and/or immunization

Hazardous chemical waste – chemicals that are potentially toxic or poisonous

All the waste generated, irrespective of type, need to be effectively disposed. The importance of proper effective waste disposal includes;

Minimizes spread of infection to health workers, patients and the local community

Reduced risk of accidental injury to those who handle the waste

Reduced likelihood of contamination of soil, ground water etc.

Reduces attraction of insects and rodents

Reduced odours

Guidelines for Disposal of Medical Waste include;

Use washable, leak-proof containers

Keep containers in convenient places

Empty containers daily or when three-quarters full

Never put hands into containers

Always dispose of medical waste correctly

Wear utility gloves and shoes

Wash the gloves and your hands afterward

Dispose off all sharps in puncture-resistant containers

Burn or bury the container of sharps

COLOUR CODING FOR WASTE SEGREGATION

Category	Example	Colour of bin
Non-infectious	Papers, food cartons,	Black
Infectious	Gloves, dressings, body fluids	Yellow
Highly infectious	Anatomical waste	Red

Figure 1.2 Colour coding for waste segregation

7.4.4 Learning Activities

7.4.4.1 Practical Activity

Task 1: Attend a home visit in your nearby health center and identify:

types of waste generated

Types of documents to keep

Ways of waste disposals

7.4.4.2 Self-Assessment

You are provided with the following questions for self -assessment, attempt them and check your responses

Why do health care workers create, disseminate and dispose patients' documents?

What colours are used in waste segregation?

Which documentations should be included in offering of psychosocial health care services?

Who are some of the outsider readers who can help themselves to patients' medical records?

What guidelines can you use for disposal of medical waste?

7.4.4.3 Tools, Equipment, Supplies and Materials

The following resources are provided:

Stationery

Note book

Gloves

Water and soap

Sanitizer

7.4.5 References

<https://onlinelibrary.wiley.com/doi/full/10.1002/pbc.25725>

<https://www.nema.go.ke/images/Docs/Media%20centre/Publication/National%20Solid%20Waste%20Management%20Strategy%20.pdf>

https://www.researchgate.net/publication/255600967_Report_Writing_Process_principles_and_styles

Responses to the self-assessment questions

Why do health care workers create, disseminate and dispose patients' documents?

facilitate provision of services later by them or by other professionals,

allow for replication of research design and analyses,

meet institutional requirements,

ensure accuracy of billing and payments, and

ensure compliance with law.

What colours are used in waste segregation?

Yellow, black and red

Which documentations should be included in offering of psychosocial health care services?

reports of patient and/or family impact of illness, of assessments conducted, nature and impact of psychosocial services provided, monitoring of illness status, and treatment plans

Who are some of the outsider readers who can help themselves to patients' medical records?

- Insurance companies ,
- Surveyors ,
- Ombudsmen,
- Consultants,
- Medicare intermediaries,
- Authorized family members

What guidelines can you use for disposal of medical waste?

- Use washable, leak-proof containers
- Keep containers in convenient places
- Empty containers daily or when three-quarters full
- Never put hands into containers
- Always dispose of medical waste correctly
- Wear utility gloves and shoes
- Wash the gloves and your hands afterward
- Dispose off all sharps in puncture-resistant containers
- Burn or bury the container of sharps